



Armada Kalamunda Group

Discharge Maternity Guideline

1. Purpose

To ensure a smooth patient journey culminating in timely discharge and communication with ongoing care providers.

2. Applicability

These guidelines apply to the midwifery and medical disciplines in the obstetrics department of Armada Health Service.

3. Guidelines

- Discharge planning is to commence during antenatal appointments, antenatal parent education classes and on admission and is to be completed on discharge.
- Utilise the Vaginal Birth Care Plan (AKMR 70.2) and Caesarean Section Care Plan (AKM 70.3) and complete education as specified.
- Medical staff to document all discharge procedures and/or requests in the patient's DMR.

3.1 Antenatal Patients

3.1.1 Antenatal Assessment Unit (AAU)

Women presenting to AAU will be assessed as per the [Antenatal Assessment Unit Inclusion Criteria Guideline](#) and discharge from AAU will be assessed and occur as per relevant AHS guideline that best reflects their admission

3.1.2 Ward

- Antenatal women will be reviewed daily by the Obstetric medical team, the decision for discharge will be determined by the Obstetric and Midwifery Team.
- Obstetric Multidisciplinary team will ensure appropriate e-referrals are completed and sent prior to discharge.
- Ensure the woman is aware of ongoing plan of care, including any further investigations, e.g. ultrasound scans and follow up antenatal appointments.
- Complete discharge summary in DMR, GPO to complete NACS, and document ongoing plan in Woman's Handheld Pregnancy Record (MR70) prior to discharge.
- Midwifery staff to ensure the Woman's Handheld Pregnancy Record (MR70) is returned to the woman.
- For women discharged back into the Maternity Group Practice (MGP) and Maternity Antenatal and Postnatal service (MAPS), inform lead midwifery carer of discharge.
- Ensure STORK has been updated with appropriate information from inpatient stay.

3.2 Postnatal Women

- Women having a caesarean section must have been medically reviewed and cleared for discharge if wanting to go home < 48hrs post birth by the Specialist Obstetrician.
- Ensure all obstetric multidisciplinary team members are happy for discharge, including Psychiatric Liaison Nurse, Physiotherapy and Social Work.
- Women with any complications e.g. Non-Elective Caesarean Section or escalation to Code Blue Responses – maternal or neonatal must have a post birth discussion with the Specialist Obstetrician, Consultant Paediatrician or the clinician involved before discharge. Ensure a six-week (or when appropriate) e-referral is submitted for an appointment in the Specialist Obstetrician clinic if deemed necessary. Preferably to the Specialist Obstetrician involved in the care.
- Ensure early referral in subsequent pregnancies for at risk patients i.e. requiring Aspirin, Progesterone in next pregnancy, early GTT in next pregnancy for GDM.
- Women with preterm birth before 35 weeks to have progesterone in next pregnancy before 16 weeks till 36 weeks, need to receive letter regarding the same along with discharge letter.
- Add Aspirin in next pregnancy if had PET, hypertension of any form in current pregnancy.

3.3 Neonate

- Ensure neonate has newborn check completed by GP Obstetrician if under care of GP Obstetrician or Paediatric team if baby under care of Paediatric team.
- Ensure that discharge check includes that baby is fit for discharge, if identified that baby requires further checks or reviews, please liaise with the Paediatric team prior to discharging the neonate, to ensure neonate is fit for discharge.
- Neonatal discharge checks to be completed in the Neonatal History and Assessment AKMR 78.1 and in the neonate's child health nurse purple book.
- Ensure any required neonatal e-referrals are completed prior to discharge and family is aware of the follow-up or referral.
- Ensure neonatal observations conducted if required - [TCB/SBR](#) Neonatal SaO₂, weight, [Newborn Hearing Screen](#) and [Newborn Blood Spot Screening](#).

3.3.1 VMS/MGP/MAPs

Ensure all women being discharged into the AHS Visiting Midwife Service (VMS) catchment resides in the AHS catchment. All women residing outside the AHS VMS catchment area please follow non-VMS patient's pathway.

- Advise woman that VMS will visit post discharge. Depending on the day of discharge women will be seen on Day 2, Day 3, Day 5 and if required Day 7. (Some women who have had a caesarean section will be seen up to Day 7, depending on the type of wound dressing applied. i.e. PICO and Pravena dressings removed Day 7.)
- Advise women under Midwifery Group Practice (MGP) or Midwifery Antenatal and Postnatal Services (MAPs) that they will be seen up to Day 10 postnatally.
- Ensure any Bentley Midwifery Birth Centre patients who were transferred for care have contact with their midwife for follow up visits when discharged.
- Visits to be scheduled to fit the clinical needs of the mother and baby and this plan communicated to the woman prior to discharge. Contact the VMS team as appropriate if there are specific rationales for timing of visits and clinical care and ensure appropriate documentation on STORK VMS Summary to provide clarity.
- Advise woman of visiting times 0830-1500hrs.

- Provide woman with the VMS/ MAPs information sheet prior to discharge and ensure they are aware of the contact number to call the Maud Bellas unit if they have any concerns.
- Complete the Postnatal Discharge Checklist – located in VMS folder on Maud Bellas.
- Check current address and phone number of women are correct for VMS to visit.
- Complete Home Visit Assessment Tool AKMR70.4
- Complete DMR discharge entry for woman and neonate
- Complete Postnatal Care Plan (AKMR 70.2/AKMR70.3) with inserted baseline set of observations, maternal wellbeing and social support details completed on VMS page.
- Complete baby's personal health record in purple book (Section Birth & Newborn: Birth details/obstetric discharge summary)
- Advise women to collect Birth Notification and Centrelink paperwork from maternity ward clerks on discharge.
- Complete STORK, print and sign:
 - VMS Summary – all instructions for care are noted in this summary in comments section and placed in plastic sleeve for VMS.
 - Copy of Child Health Services – Place in the purple book
 - General Practitioner copy will be mailed out to the GP by the ward clerk. If the woman does not have a GP- give a copy to the woman.
 - If a Special Child Health Nurse referral required – email via STORK. See below for requirements to complete SCHN referral.
- Notify VMS, MGP or MAPs midwife if change in discharge plan or special precautions necessary for home visit
- Place in plastic sleeve the Home Visit Assessment Tool AKMR70.4, Stork Visiting Midwifery Referral, Vaginal Care Plan AKMR 70.2 or Caesarean Section Care Plan AKMR 70.3, Neonatal History and Assessment AKMR 78.1 and 1 sheet of Mother's ID address labels, 1 sheet of Baby's ID address labels. Place plastic sleeve and documents into VMS tray/MGP tray or MAPs tray.

3.3.2 Early Discharge

Women eligible for medically cleared postnatal discharge at 6 hours post birth must meet the following criteria;

- Low risk pregnancy
- Nil complications intrapartum
- Nil neonatal risk factors
- Neonatal Oxygen saturations within 1 hour of discharge with a reading >95%
- GP Obstetrician performed baby check – nil abnormalities found

If all criteria met;

- Complete STORK, print and sign:
 - VMS Summary – all instructions for care are noted in this summary in comments section
 - Copy of Child Health Services – Place in the purple book
 - General Practitioner copy will be mailed out to the GP by the ward clerk. If the woman does not have a GP- give a copy to the woman.
- Commence Postnatal Care Plan (AKMR 70.2) with inserted baseline set of observations, maternal wellbeing and social support details.

- Commence Neonatal History and Assessment (AKMR 78.1)
- Complete baby's personal health record in purple book (section Birth & Newborn: Birth details/obstetric discharge summary)
- Follow VMS/MGP/MAPs process as above

3.3.3 Non-VMS patients

- Those women who are out of our VMS catchment:
 - Complete documentation and STORK as above for VMS
 - If for VMS in another catchment refer to VMS folder for instructions on referral processes for each hospital
- Women who wish to return to the ward for care as outpatient
 - Complete documentation and STORK as above for VMS
 - Ensure medical record is kept in VMS drawer of filing cabinet and ensure handover of woman's attendance noted

3.3.4 Postnatal woman not wishing to have follow up

- Complete documentation as above
- Women discharging against medical advice, who either decline or are not suitable for VMS, ensure Special Child Health Nurse referral via STORK is completed and emailed

3.3.5 Baby who has been under the care of Paediatrician

- Ensure baby has been discharged by paediatrician and all relevant follow up has been organised

3.3.6 Special Child Health Nurse referrals

To ensure the effective transfer of information from maternity hospitals and neonatal services to child health services for families with known newborn or family risk factors identified in the antenatal or postnatal period midwives must ensure appropriate referral utilising STORK

- Complete a Special Child Health Nurse referral via STORK for risk factors detailed in the table below, ensuring that the referral is emailed and printed via STORK
- In the event of a neonate being readmitted and risk factors identified, another Special Child Health Nurse referral is to be generated, emailed and printed via STORK
- Summary of Care/Additional Information re Referral to be documented in STORK 'Notes' section (include discharge medication) prior to emailing and printing

3.3.7 Identified Risk Factors/Reason for Special Child Health Nurse referral

Reasons for Special CHN:	
Family Domestic Violence	FDV identified, referral accepted, FDV identified, referral declined
EPDS at Booking or Trimester 3	Currently receiving psychological care, Referral to psychological medicine, Referral to social work, Score 9-11 GP informed antenatally
Alcohol < or > 20 weeks	Response in Pregnancy Health Review is Drinks Containing Alcohol - Monthly, Most Days each Week, Some Days each Week, Weekly (NB: EXCLUDES Drinks Containing Alcohol - Occasional/Infrequent eg. Once or Twice in Pregnancy, Undetermined/Unknown or No Alcohol in Pregnancy)
Substance Use in Pregnancy	If recorded in Pregnancy Health Review
WANDAS as AN Care Team	If recorded in Antenatal Care Teams
Anxiety, depression, mental illness	Anxiety or depression or other mental illness (Psychiatric disorder - unspecified, psychological issues, bipolar affective disorder, Schizoaffective Disorder, Schizophrenia) recorded as Medical Condition
Boodjari Yorgas & Koolanga Involvement	If recorded in Postnatal Follow Up Arrangements
Department for Child Protection Involvement	If recorded in Postnatal Follow Up Arrangements
Parent has Intellectual Disability	If 'Intellectually Challenged' is recorded in Pregnancy Medical Conditions
Indication for Foster Care/Adoption	If Baby Outcome is 'Other' NB: Adoption no longer recorded in Stork
Maternal Morbidity eg PPH	If 3rd or 4th Degree tear (with or without episiotomy) recorded in Perineal Trauma
Maternal Morbidity Perineal trauma	If Total Blood Loss >= 500
Infant has disability	If 'Cardiac Condition, Hypothyroidism, Seizures' recorded in Baby Morbidity
Infant has difficulties feeding	If 'Feeding Difficulties' is recorded in Baby Morbidity

Parent is Homeless	If 'Homeless currently' is recorded in Pregnancy Health Review - Accommodation
Parent is Teenager	If Maternal age at date of birth of baby <= 18 NB: unable to determine if low income or unsupported
Reasons for Special CHN: Automatic Trigger Items - these are not included in this report - refer to report 'Special CHN Referral not Emailed'	
Infant Stillbirth or Neonatal Death	
Infant of multiple birth	
Infant indeterminate gender	
Infant weight loss >10% of birth weight	
Infant low birth weight	
Infant transfer to Special Care Nursery	
Infant prematurity	

Other agencies involved/referred

Prior to discharge consider if referral required and ensure this is sent, prior to discharge.

☐ Social Worker	☐ Paediatrician
☐ Mental Health Services	☐ Hospital Home Visiting Service
☐ Referred to Department of Child Protection	☐ Mother Baby Unit
☐ Referred to Best Beginnings Enhanced Home Visiting Services	☐ Lactation Consultant
☐ Other	☐ Specialist Obstetrician
☐ Physiotherapy	☐ Continence Nurse

4. Legislative context

The following documents are required to give effect to this policy:

- [WA Health Specialist Outpatient Services Access Policy od 0530/14](#)
- [Patient Discharge- AKG Policy](#)

5. Supporting information

The following documents support the implementation of this policy:

- [National Midwifery Guidelines for Consultation and Referral - 4th Edition, \(2021\)](#)
- [WA Health Safe infant Sleeping Policy Framework OD 0474/13](#)
- [WA Health Newborn Hearing Screening- Policy Frameworks OD 0239/09](#)

6. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.6
- Standard 2 – Action 2.10

8. Document owner

Enquiries relating to this guideline may be directed to:

Title: Midwifery Manager
 Department: Maternity
 Email: Shannon.scari@health.wa.gov.au

9. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.0	13/08/2025	13/08/2028	Section 3.1.2 – Obstetric and Midwifery Team GPO to complete NACS 3.2 = ‘a post birth discussion’ Ensure early referral in subsequent pregnancies for at risk patients i.e. requiring Aspirin, Progesterone in next pregnancy, early GTT in next pregnancy for GDM. Add Aspirin in next pregnancy if had PET, hypertension of any form in

			current pregnancy. Add Aspirin in next pregnancy if had PET, hypertension of any form in current pregnancy. 3.3.1 - (Some women who have had a caesarean section will be seen up to Day 7, depending on the type of wound dressing applied, i.e. PICO and Pravena dressings removed Day 7.) Ensure any Bentley Midwifery Birth Centre patients who were transferred for care have contact with their midwife for follow up visits when discharged. Advise women to collect Birth Notification and Centrelink paperwork from maternity ward clerks on discharge. Place plastic sleeve and documents into VMS tray/MGP tray or MAPs tray.
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10. Authorisation

This guideline has been authorised and issued by the Director of Clinical Services.

Authorised by	Tania Strickland – A/Director of Clinical Services
Authorisation date	08/08/2025
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